

Insurance Utilization and Claims Behavior Study

Executive Summary

This report presents an analysis of insurance utilization patterns, claim behaviors, and operational challenges observed within the healthcare system. The purpose of this study is to understand common reasons for claim rejections, underutilization of insurance coverage, process inefficiencies, and patterns of high-cost disease claims.

The analysis highlights administrative gaps, patient awareness issues, documentation errors, and approval delays that impact both patient experience and financial outcomes. It also examines trends in high-value claims and areas where fraud suspicion or claim irregularities have been observed.

Overview of Insurance Utilization Trends

Insurance coverage is intended to reduce financial burden and improve access to healthcare services. However, data indicates that a significant number of patients either underutilize their coverage or face repeated claim rejections.

Despite being insured, many patients do not claim the full benefits available to them. Some patients choose to self-pay partially even when eligible for reimbursement. These trends indicate the presence of structural, informational, and procedural barriers within the claim ecosystem.

Reasons for Claim Rejection

Claim rejections occur due to a combination of administrative, procedural, and eligibility-related factors.

One of the most common reasons for rejection is incomplete or inaccurate documentation. Missing signatures, unsigned discharge summaries, mismatched policy numbers, and incorrect diagnostic coding frequently result in claim denial.

Non-covered treatments also contribute significantly to rejection rates. Many patients undergo procedures or receive medications that are not included within their policy coverage. This is particularly common in cases involving advanced therapies, cosmetic procedures, experimental treatments, or room upgrades beyond policy limits.

Late submission of claims is another recurring issue. Insurance providers typically enforce strict submission timelines. Claims submitted beyond the permitted window are often automatically rejected, regardless of treatment necessity.

Pre-authorization failures also lead to denials. In certain high-cost procedures, prior approval is mandatory. Failure to obtain timely authorization results in claim non-approval.

Documentation Errors and Administrative Gaps

Documentation-related errors are among the most significant contributors to claim rejection.

Common issues include:

- Incorrect patient details
- Policy number mismatches
- Incomplete discharge summaries
- Missing physician signatures
- Lack of itemized billing
- Coding discrepancies

Hospitals sometimes submit handwritten or unclear documentation, leading to processing delays or rejection. Inadequate coordination between billing departments and clinical teams also increases the likelihood of documentation gaps.

Additionally, lack of standardized documentation formats across institutions creates inconsistencies in claim processing.

Non-Covered Treatments and Policy Limitations

Many claim rejections stem from misunderstandings regarding policy inclusions and exclusions.

Certain treatments are excluded under standard insurance policies, including:

- Cosmetic procedures
- Experimental therapies
- Alternative or non-evidence-based treatments
- Non-medical consumables
- Certain high-end implants

Patients are often unaware of these exclusions at the time of admission. As a result, disputes arise after discharge when claims are partially approved or rejected.

Room rent capping is another common limitation. If a patient selects a room category exceeding policy entitlement, proportionate deductions are applied across the total bill.

Late Submission Patterns

Late claim submission is frequently observed in reimbursement-based policies.

Common contributing factors include:

- Patient unawareness of submission deadlines
- Delay in obtaining final discharge documents
- Incomplete bill settlement paperwork
- Lack of follow-up by patients

Many patients assume hospitals will automatically process claims in reimbursement cases. However, in such models, responsibility often lies with the patient, leading to missed deadlines.

Why Patients Do Not Use Full Insurance Coverage

A significant number of insured patients do not fully utilize their policy limits. Several behavioral and psychological factors contribute to this trend.

Lack of Awareness

Many patients are unaware of their policy coverage limits, inclusions, and entitlements. They may not know about daycare procedures, post-hospitalization benefits, or preventive coverage options.

Fear of Premium Increase

Some patients avoid making claims due to fear that their insurance premium will increase during renewal. They prefer to pay smaller medical bills out-of-pocket to maintain perceived long-term financial benefit.

Complex Claim Process

The insurance claim process is often viewed as complicated and time-consuming. Lengthy forms, multiple verification steps, and documentation requirements discourage patients from claiming smaller amounts.

Perceived Low Approval Probability

Patients who previously experienced rejection may feel discouraged and choose not to claim again.

High-Cost Disease Claim Patterns

Analysis indicates that certain diseases consistently generate higher claim values.

These include:

- Cancer
- Cardiac diseases (bypass surgery, angioplasty)
- Chronic kidney disease (dialysis, transplant)
- Major trauma cases
- Neurological surgeries

Cancer-related claims are particularly high due to prolonged treatment cycles, chemotherapy, targeted therapies, and repeated hospital admissions. Cardiac interventions also generate large claim volumes due to device costs and intensive care requirements. Chronic diseases requiring long-term hospitalization contribute to cumulative high-value claims.

Fraud Suspicion Commentary

While the majority of claims are genuine, certain patterns raise fraud suspicion indicators.

These include:

- Repeated high-value claims from the same beneficiary
- Inflated billing amounts
- Unusual frequency of admissions
- Mismatch between diagnosis and treatment cost
- Duplicate claim submissions

Insurance providers often conduct additional verification in such cases, leading to extended approval timelines.

It is important to note that suspicion does not imply confirmed fraud but triggers further review.

Delays in Approval – Observations

Claim approval delays are frequently reported by both hospitals and patients.

Common reasons include:

- High claim volume
- Incomplete documentation
- Additional clarification requests
- Internal insurer verification processes
- Query cycles between hospital and insurer

Delays are more common in high-cost claims requiring medical board review.

Such delays impact patient discharge processes and hospital cash flow management.

Process Issues Identified

Several systemic issues contribute to inefficiencies in insurance utilization:

- Lack of standardized documentation systems
- Poor communication between hospital and insurer
- Insufficient patient counseling at admission
- Manual paperwork dependency
- Limited digital integration

Improved digitization and pre-admission counseling could significantly reduce rejection and delay rates.

Overall Observations

The study indicates that claim rejection and underutilization are largely driven by:

- Administrative errors
- Policy misunderstanding

- Behavioral hesitation
- Process complexity
- Documentation gaps

High-cost diseases such as cancer and cardiac conditions generate disproportionate claim volumes, requiring stricter scrutiny and longer approval timelines.

Improving transparency, simplifying processes, and enhancing patient education can significantly improve insurance utilization efficiency.

Patient Assistance & Financial Policy Framework

Copay Assistance Eligibility

Copay assistance programs are designed to reduce patient out-of-pocket costs for branded medications. However, eligibility is strictly regulated.

Only patients enrolled in **Commercial Insurance plans** are eligible for manufacturer-sponsored copay assistance programs. Patients covered under federal healthcare programs, including Medicare, Medicaid, and TRICARE, are legally excluded from receiving such subsidies. This restriction ensures compliance with federal Anti-Kickback Statutes, which prohibit financial incentives that may influence prescribing or purchasing decisions within government-funded programs.

As a result, a segment of patients may face higher financial burdens despite needing therapy, particularly those transitioning from commercial plans to government-funded coverage.

Denial Conditions for Financial Assistance

Financial assistance or copay support may be denied under the following conditions:

1. The patient is enrolled in a government-funded insurance plan.

2. The prescription is written for an off-label use not approved by regulatory authorities.
3. The patient's household income exceeds 500% of the Federal Poverty Level (FPL).

Income-based eligibility screening is commonly used to allocate limited assistance resources to financially vulnerable populations. However, this may create access gaps for middle-income patients who do not qualify for assistance but still experience financial strain.

Impact of High-Deductible Health Plans

Patients enrolled in high-deductible health plans (HDHPs) are required to pay the negotiated rate of the medication in full until their annual deductible is met.

In such cases:

- Copay subsidies cannot be applied before the primary insurance claim is processed.
- After adjudication, copay support may be used to reduce (“buy down”) the remaining out-of-pocket responsibility.

This structure often results in significant upfront financial burden early in the policy year, leading some patients to delay therapy initiation or discontinue treatment due to cost concerns.

Clinical Operations & Risk Management (REMS Framework)

Adherence Thresholds

Medication adherence is monitored using the Proportion of Days Covered (PDC) metric.

A PDC below 0.80 is classified as Non-Adherent. Patients below this threshold are considered at increased risk of suboptimal treatment outcomes and potential disease progression.

Monitoring adherence is essential for both clinical effectiveness and payer compliance.

Behavioral Nudge Protocol

To address declining adherence, a structured intervention model is implemented.

If a patient's PDC falls between 0.50 and 0.79, a "Step 1 Nudge" is triggered. This includes automated SMS medication reminders and a pharmacist follow-up call.

If the PDC falls below 0.50, a more intensive "Step 2 Intervention" is initiated, requiring a home health nurse visit for patient counseling and therapy reinforcement.

This escalation model ensures early behavioral correction before full treatment discontinuation occurs.

Side Effect Monitoring & Safety Reporting

Under the Risk Evaluation and Mitigation Strategy (REMS) framework, adverse event monitoring is mandatory.

If more than 15% of the active patient population reports severe fatigue or dizziness, the Brand Team is required to issue a Physician Safety Update within 30 days. This ensures timely communication of safety signals to prescribing clinicians.

Such monitoring mechanisms aim to protect patient safety while maintaining regulatory compliance.

Treatment Failure Definition

Treatment failure is defined as:

- Lack of measurable clinical response after 90 days of continuous therapy, or
- Occurrence of a Grade 3 adverse event requiring permanent discontinuation.

This standardized definition ensures consistency in therapy evaluation and supports clinical decision-making regarding therapy modification or escalation.

Market Access & Enrollment Barriers

Prior Authorization (PA) Requirements

Prior Authorization is a utilization management tool used by health plans to control drug costs.

Before coverage approval, providers must submit documentation demonstrating medical necessity. If a PA is pending, enrollment coordinators are responsible for collecting:

- Physician's Statement of Medical Necessity (SMN)
- Documentation of previous step therapy attempts
- Clinical history supporting treatment need

Delays in PA submission or incomplete documentation significantly contribute to therapy initiation delays and claim processing issues.

Step Therapy (“Fail-First” Policy)

Step Therapy policies require patients to attempt lower-cost generic alternatives before a high-cost branded medication is approved.

For example, patients may be required to trial generic statins before approval of a branded therapy such as Zynexis-A.

While this approach controls payer expenditure, it can delay optimal therapy initiation and may result in suboptimal disease control during the trial period.

Patient Access Workflow – Payer Approval Delays

Delays in payer approval most frequently occur during the prior authorization stage of the access workflow. The most common delay arises from incomplete clinical documentation submitted with the initial request, particularly when supporting notes fail to clearly

demonstrate alignment with payer criteria. Incorrect diagnostic coding and mismatched ICD submissions often trigger automatic review flags, leading to resubmissions and extended turnaround times. Missing laboratory values, absent trial history documentation, and incomplete step therapy records further contribute to administrative back-and-forth between provider offices and payers. Clinical criteria misalignment, especially when prescribers are unaware of updated plan requirements, results in avoidable denials and appeal cycles. Appeals are frequently triggered not because the therapy lacks merit, but due to incomplete or improperly structured submissions. Repeated PA form resubmissions significantly extend time-to-approval and delay therapy initiation.

Financial Assistance – Eligibility & Denial Patterns

Financial assistance requests are most commonly denied due to eligibility restrictions tied to insurance type and income thresholds. Patients enrolled in government-funded programs are typically ineligible for manufacturer-sponsored copay support due to federal compliance regulations. Income threshold exceedance is another leading cause of denial, particularly when household earnings surpass defined assistance program limits. Missing or incomplete income verification documentation frequently delays approval or results in temporary rejection. Residency requirements also play a role, as certain assistance programs are limited to specific geographic regions. Coverage duplication, where a patient already receives alternate subsidy support, may disqualify additional assistance. Incomplete application packets, including unsigned forms or missing provider certification, further contribute to denial patterns. These combined regulatory and documentation factors significantly influence financial access outcomes.

Insurance Coverage Landscape – Restrictive Policies

Restrictive payer policies increasingly shape therapy access conditions across commercial and government plans. Step therapy mandates remain one of the most significant barriers, requiring documented failure on lower-cost alternatives before authorization of branded therapies. Fail-first protocols often demand evidence of trial on one or more preferred agents prior to coverage consideration. Placement on Tier 4 or Tier 5

specialty formularies increases patient cost exposure and may discourage therapy initiation. Prior authorization renewal frequency requirements also create ongoing administrative burden, requiring periodic revalidation of medical necessity. Quantity limits and dosing caps further restrict prescribing flexibility, particularly in chronic therapy management. Age restrictions, diagnosis specificity requirements, and site-of-care mandates (such as infusion center restrictions) add additional layers of access complexity. For example, several Medicare Advantage plans now require documented failure on two preferred agents before approving specialty-tier therapy.